

PAC2511-0003
Leaflet - Zynor Tablet 100BP (HK Tender)
Size: 148(L) x 210(H)mm
Color: Black

Important information. Please read carefully.



COMPOSITION

Zynor 5mg: Each tablet contains Amlodipine Besylate equivalent to Amlodipine 5 mg
Zynor 10mg: Each tablet contains Amlodipine Besylate equivalent to Amlodipine 10 mg

PHARMACODYNAMICS

Amlodipine is a calcium ion influx inhibitor of the dihydropyridine group (slow channel blocker or calcium ion antagonist) and inhibits the transmembrane influx of calcium ions into cardiac and vascular smooth muscle.

The mechanism of the antihypertensive action of amlodipine is due to a direct relaxant effect on vascular smooth muscle. The precise mechanism by which amlodipine relieves angina has not been fully determined but amlodipine reduces total ischaemic burden by the following two actions:

1. Amlodipine dilates peripheral arterioles and thus, reduces the total peripheral resistance (afterload) against which the heart works. Since the heart rate remains stable, this unloading of the heart reduces myocardial energy consumption and oxygen requirements.

2. The mechanism of action of amlodipine also probably involves dilatation of the main coronary arteries and coronary arterioles, both in normal and ischaemic regions. This dilatation increases myocardial oxygen delivery in patients with coronary artery spasm (Prinzmetal's or variant angina).

In patients with hypertension, once daily dosing provides clinically significant reductions of blood pressure in both the supine and standing positions throughout the 24 hour interval. Due to the slow onset of action, acute hypotension is not a feature of amlodipine administration.

In patients with angina, once daily administration of amlodipine increases total exercise time, time to angina onset, and time to 1mm ST segment depression, and decreases both angina attack frequency and nitroglycerine tablet consumption.

Amlodipine has not been associated with any adverse metabolic effects or changes in plasma lipids and is suitable for use in patients with asthma, diabetes, and gout.

PHARMACOKINETICS

Absorption, distribution, plasma protein binding: After oral administration of therapeutic doses, amlodipine is well absorbed with peak blood levels between 6-12 hours post dose. The absorption of amlodipine is unaffected by the concomitant intake of food. Absolute bioavailability has been estimated to be between 64 and 80%. The volume of distribution is approximately 21 l/kg. *In vitro* studies have shown that approximately 97.5% of circulating amlodipine is bound to plasma proteins.

Biotransformation/elimination: The terminal plasma elimination half life is about 35-50 hours and is consistent with once daily dosing. Steady state plasma levels are reached after 7-8 days of consecutive dosing. Amlodipine is extensively metabolised by the liver to inactive metabolites with 10% of the parent compound and 60% of metabolites excreted in the urine.

Use in the elderly: The time to reach peak plasma concentrations of amlodipine is similar in elderly and younger subjects. Amlodipine clearance tends to be decreased with resulting increases in AUC and elimination half-life in elderly patients. Increases in AUC and elimination half-life in patients with congestive heart failure were as expected for the patient age group studied.

INDICATIONS

Hypertension: Zynor is indicated for the first line treatment of hypertension and can be used as the sole agent to control blood pressure in the majority of patients. Patients not adequately controlled on a single antihypertensive agent may benefit from the addition of Zynor, which has been used in combination with a thiazide diuretic, alpha blockers, beta adrenoceptor blocking agent, or an angiotensin converting enzyme inhibitor.

Myocardial Ischemia: Zynor is indicated for the first line treatment of myocardial ischemia, whether due to fixed obstruction (stable angina) and/or vasospasm/vasoconstriction (Prinzmetal's or variant angina) of coronary vasculature. Zynor may be used where the clinical presentation suggests a possible vasospastic/vasoconstrictive component but vasospastic/vasoconstrictive has not been confirmed. Zynor may be used alone, as monotherapy, or in combination with other antianginal drugs in patients with angina that is refractory to nitrates and/or adequate doses of beta blockers.

DOSAGE AND ADMINISTRATION

Adults: For both hypertension and angina the usual initial dose is 5 mg Zynor once daily which may be increased to a maximum dose of 10 mg depending on the individual patient's response.

No dose adjustment of Zynor is required upon concomitant administration of thiazide diuretics, beta blockers and angiotensin-converting enzyme inhibitors.

In children: Not recommended.

In the elderly: Normal dosage regimens are recommended. Amlodipine, used at similar doses in elderly or younger patients, is equally well tolerated.

Patients with hepatic impairment: See section Precautions.

Patients with renal impairment: Amlodipine may be used in such patients at normal doses. Changes in amlodipine plasma concentrations are not correlated with degree of renal impairment. Amlodipine is not dialyzable.

CONTRAINDICATIONS

Amlodipine is contra-indicated in patients with a known sensitivity to dihydropyridines, amlodipine or any of the excipients.

Amlodipine should not be used in cardiogenic shock, clinically significant aortic stenosis, unstable angina (excluding Prinzmetal's angina).

WARNINGS AND PRECAUTIONS

General: Since the vasodilation induced by amlodipine is gradual in onset, acute hypotension has rarely been reported after oral administration. Nonetheless, caution, as with any other peripheral vasodilator, should be exercised when administering amlodipine, particularly in patients with severe aortic stenosis.

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Use in Patients with Heart Failure: Patients with cardiac failure should be treated with caution. In a long-term, placebo controlled study in patients suffering from severe heart failure (NYHA III and IV) of nonischaemic aetiology, amlodipine was associated with increased reports of pulmonary oedema despite no significant difference in the incidence of worsening heart failure as compared to placebo.

Patients with Hepatic Failure: As with all calcium antagonists, amlodipine's half life is prolonged in patients with impaired hepatic function and dosage recommendations have not been established. Caution should be exercised when administering amlodipine to these patients.

Effects on Ability to Drive and Use Machines: Clinical experience with amlodipine indicates that therapy is unlikely to impair a patient's ability to drive or use machinery.

DRUG INTERACTIONS

Amlodipine has been safely administered with thiazide diuretics, alpha blockers, beta blockers, angiotensin-converting enzyme inhibitors, long-acting nitrates, sublingual glyceryl trinitrate, non-steroidal anti-inflammatory drugs, antibiotics, and oral hypoglycaemic drugs.

In vitro data from studies with human plasma indicate that amlodipine has no effect on protein binding of digoxin, phenytoin, warfarin, or indomethacin.

In healthy volunteers, the co-administration of amlodipine does not significantly alter the effect of warfarin on prothrombin response time.

Pharmacokinetic studies with cyclosporin have demonstrated that amlodipine does not significantly alter the pharmacokinetics of cyclosporin.

PREGNANCY AND LACTATION

Safety of amlodipine in human pregnancy or lactation has not been established. Amlodipine does not demonstrate toxicity in animal reproductive studies other than to delay parturition and prolong labor in rats at dose level fifty times the maximum recommended dose in humans. Accordingly, use in pregnancy is only recommended when there is no safer alternative and when the disease itself carries greater risk for the mother and fetus.

SIDE EFFECTS / ADVERSE REACTIONS

Amlodipine is well tolerated. In placebo controlled clinical trials involving patients with hypertension or angina, the most commonly observed side effects were headache, oedema, fatigue, somnolence, nausea, abdominal pain, flushing, palpitations, and dizziness. In these clinical trials, no pattern of clinically significant laboratory test abnormalities related to amlodipine has been observed.

Less commonly observed side effects in marketing experience include alopecia, altered bowel habits, arthralgia, asthenia, back pain, dyspepsia, dyspnea, gingival, hyperplasia, gynecostasia, hyperglycemia, impotence, increased urinary frequency, leucopenia, malaise, mood changes, dry mouth, muscle cramps, myalgia, peripheral neuropathy, pancreatitis, increased sweating, syncope, thrombocytopenia, vasculitis, and visual disturbances. In many instances, causal association is uncertain. Rarely, allergic reaction including pruritus, rash, angioedema, and erythema multiforme.

Hepatitis, jaundice and hepatic enzyme elevations have also been reported very infrequently (most consistent with cholestasis). Some cases severe enough to require hospitalization have been reported in association with use of amlodipine. In many instances, causal association is uncertain.

As with other calcium channel blockers the following adverse events have been rarely reported and cannot be distinguished from the natural history of the underlying disease: myocardial infarction, arrhythmia (including ventricular tachycardia and atrial fibrillation) and chest pain.

SYMPTOMS AND TREATMENT FOR OVERDOSAGE

Available data suggest that gross overdosage could result in excessive peripheral vasodilatation and possibly reflex tachycardia. Marked and probably prolonged systemic hypotension up to and including shock with fatal outcome have been reported.

Gastric lavage may be worthwhile in some cases. Clinically significant hypotension due to amlodipine overdosage calls for active cardiovascular support including frequent monitoring of cardiac and respiratory function, elevation of extremities, and attention to circulating fluid volume and urine output. A vasoconstrictor may be helpful in restoring vascular tone and blood pressure, provided that there is no contraindication to its use. Intravenous calcium gluconate may be beneficial in reversing the effects of calcium channel blockade. Since amlodipine is highly protein-bound, dialysis is not likely to be of benefit.

STORAGE CONDITION : Store below 30°C

KEEP ALL MEDICINES OUT OF REACH OF CHILDREN/JAUHI DARI KANAK-KANAK

SHELF LIFE: The expiry date is indicated on the packaging.

PRODUCT DESCRIPTION:

Zynor 5mg: White color, oval shape (8.5mm x 5.0mm) normal convex tablet, engraved with "A5" and break line on one side and plain on the reverse. P/VdC/Alu blister pack of 10's in packing of 30's, 100's and 1000's per box. Not all pack size(s) are available for sale on the market.

Zynor 10mg: White colour, oval shape (11.0mm x 6.0mm) normal convex tablet, engraved with "X5" and break line on one side and plain on the reverse. P/VdC/Alu blister pack of 10's in packing of 30's, 100's and 1000's per box. Not all pack size(s) are available for sale on the market.

For further information, please consult your pharmacist or physician.

Revision Date : 20-Apr-2026

Manufacturer and Product Registration Holder (in Malaysia)

Xepa-Soul Pattinson (Malaysia) Sdn. Bhd.

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